AI Technical Program Manager — UST

Week 1 · Exercise 2 · Case Pack

Kestrel Health Network — "Beacon" AI-assisted patient triage

For: every participant. Individual exercise.

Work product: Release-Readiness Checklist v1 for Beacon, submitted in the Exercise 2 Answer Workbook

Suggested time: 3 to 4 hours. Use any AI tool you like; you are assessed on judgment, not tooling.

Scored against: the same Week 1 rubric you already know (three core skills and the Trust & Governance Design competency)

Before you begin: why this document exists

Why this step is needed

In the Meridian lab you learned to see through a checklist that looked complete. Real releases are rarely that obvious. This exercise gives you a team that has already learned some of the lessons: its checklist names people, has a stage column and records gate approvals. The question is whether any of that is real. You need to practise finding the Completeness Illusion when it is well disguised.

What exactly happens in this step

You join Kestrel Health Network as AI Technical Program Manager two days before the Go/No-Go meeting for Beacon, an AI system that triages patient messages. You read the briefing, the policies, the inherited checklist v0.9 and an evidence pack. You then complete the Exercise 2 Answer Workbook: a Findings Log, a governance frame with both stage gates and phase gates, a Release-Readiness Checklist v1, and a readiness statement that gives the sponsor a clear decision for the 14 December pilot.

The purpose of what is being done

To test whether you can apply the Week 1 toolkit (four questions, five dimensions, three AI controls, six stages) without hints, in an unfamiliar domain, against subtler evidence, and to make a release call that a regulator could follow.

Which part of the programme architecture this builds

This is an additional Week 1 exercise in the six-week programme. It strengthens the foundation layer (release readiness) and exercises the same competency, Trust & Governance Design, that Week 2 assesses at a higher target level. It also gives you practice in a second domain track (Healthcare), which supports the Cross-Domain Range expected later in the programme.

Part A — Your briefing

Your role

You have joined Kestrel Health Network today as the **AI Technical Program Manager** for Beacon. Today is **Tuesday 8 December 2026**. The pilot Go/No-Go meeting is on **Thursday 10 December 2026**. The pilot is due to go live on **Monday 14 December 2026** at two hospitals, and across all nine hospitals on **Monday 11 January 2027**.

The sponsor, Dr. Anjali Rao, has asked you for one thing: *"Give me a straight answer on the 14th, and tell me what would make it a yes."*

You may use any AI platform or tool to help you read, analyse and draft. You are assessed on the quality of your judgment and the trustworthiness of your checklist, never on which tool you used.

A1. Kestrel Health Network

Kestrel is a private hospital network with nine hospitals and about 2.1 million registered patients. Patients contact Kestrel in two ways that matter here: **GP referral letters**, sent by family doctors, and **patient-portal messages**, written by patients themselves, often on a phone, in English, Hindi, Marathi or a mix of these written in Latin script.

Today every message is read by a triage nurse, who decides how urgent it is. The busiest sites take several days to answer routine messages, and nurses are stretched at night.

A2. Beacon — the product

Beacon is an AI-assisted triage system. For every incoming referral letter or portal message it:

1. Reads the message and assigns an urgency category, **Emergency, Urgent, Routine or Self-care**, with a confidence score and a short rationale for the nurse.
2. Sends every Emergency and Urgent message to the triage nurse queue.
3. For Routine and Self-care messages with confidence **above the auto-route threshold**, routes them automatically: Routine messages go to the right clinic's booking list; Self-care messages receive an **automated reply** with self-care advice drafted by Beacon.
4. Sends everything below the threshold to the nurse queue.

Beacon calls a large language model hosted by **Corvid AI** through an API, and connects to Kestrel's systems through **MedLink Systems**. Most of the code was written with agentic coding tools.

Two-phase rollout

Phase	Scope	Date
Pilot	Kestrel Riverside and Kestrel Hillcrest: referral letters and portal messages, automatic routing and automated replies switched on	Monday 14 December 2026
Network rollout	All nine hospitals, same configuration	Monday 11 January 2027

A3. The people

These are the people you may assign as owners. You may not invent people, and you may not assign a team, a supplier, or someone who has left.

Name	Role	What you should know
Dr. Anjali Rao	Chief Medical Officer — Programme Sponsor	Sponsors Beacon. Chairs the Go/No-Go on 10 Dec. Has asked you: "Give me a straight answer on the 14th, and tell me what would make it a yes."
Marcus Bell	Chief Digital Officer	Owns the business case and the budget. The 14 Dec pilot was announced in the board's annual report.
Neha Sharma	Delivery Lead, Beacon	Runs the delivery team. Wrote checklist v0.9. Approved the stage gates recorded in it.
Tobias Grant	Solution Architect (contractor)	Designed the integration architecture. His contract ends on 18 December 2026.
Imran Qureshi	Lead Engineer / Agent Orchestrator	Runs the agentic coding workflow: writes the prompts and tasks the coding agents work from, and approves pull requests.
Dr. Helen Mathews	Clinical Safety Officer (CSO)	Owns Kestrel's clinical safety standard. Has not yet reviewed Beacon's safety case.
Priyanka Das	Head of Nursing	Manages the triage nurses who receive cases Beacon does not route automatically.
Victor Osei	QA Manager	Joined Kestrel in November 2026. Owns the test strategy from that date.
Dr. Farah Siddiqui	Head of Data & AI	Built Kestrel's evaluation set and ran the internal evaluation. Proposed the 0.80 threshold.
Rahul Menon	Chief Information Security Officer (CISO)	Owns security testing and sign-off.
Grace Liu	Data Protection Officer (DPO)	Owns the Data Protection Impact Assessment (DPIA).
Owen Fletcher	Head of Risk & Compliance	Owns the AI Use Policy (AIP-2) and the enterprise risk register.
Kavya Nair	Platform & SRE Lead	Owns hosting, monitoring, alerting and the platform on-call rota.
Sunil Bhat	Patient Contact Centre Manager	His team handles patient phone calls and the general patient inbox.

No longer with Kestrel

Name	Role	Note
Jonas Weber	Former QA Lead	Left Kestrel on 16 October 2026.

Suppliers (not people you can assign as owners)

Supplier	Provides	Note
Corvid AI	Large language model provider	Hosts the triage model behind an API. Not a Kestrel employee.
MedLink Systems	Patient-record integration vendor	Connects Beacon to the patient portal and the referral system.

A4. Kestrel policies that apply

Policy	What it requires
AIP-2 · AI Use Policy (owner: Owen Fletcher)	Clinical risks from AI systems may be accepted only by the Chief Medical Officer or the Clinical Safety Officer, in writing, with a review date. Any change to a model, prompt or threshold that affects clinical output requires re-evaluation on Kestrel data and CSO approval before use.
CSS-1 · Clinical Safety Standard (owner: Dr. Helen Mathews)	Every system that influences patient care needs a Kestrel clinical safety case and hazard log, owned and signed by Kestrel's CSO. A supplier's hazard log is an input to it, never a substitute.
RP-1 · Records Policy	Patient communications, and the record of every decision made about a patient's care, are kept for 8 years.
PCS-3 · Patient Communication Standard	Written messages to patients use a reading level of grade 8 or below, and are offered in the patient's chosen language (English, Hindi or Marathi). Patients must be told when a message was drafted by an automated system.

A5. The artefact chain today

Stage	Artefact	State today
Intent	beacon/intent.md	Written in August by Marcus and Neha. Covers triage of GP referral letters only. On 9 October patient-portal messages and automated replies were added to scope by a decision in the team channel; intent.md was not updated.
Spec	beacon/spec.md v3	Updated in October for portal messages by Imran using a coding agent. The clinical safety section is copied from Corvid's hazard log. Approved by Neha on 2 December.
Plan	beacon/plans/ (one per feature)	Plans exist for 9 of 11 features. "Automated reply drafting" and "threshold tuning" have no plan; Imran describes them as "small config features".
Build	Pull requests, CI runs, evaluation runs	87 PRs merged; about 70% written by coding agents. Current build 1.4.2. Internal evaluation ev-118 ran on 12 November on build 1.4.0.
Review	Review-agent reports; human approvals	Every PR has a review-agent report. 12 PRs were approved by Imran, who also wrote the prompts the coding agent used to produce them.
Deploy	Pipeline logs; staging; monitoring	Running in staging since 18 November. Pilot configuration ready for the two sites. A "Beacon Drift" dashboard exists (see evidence E5).

Part B — The inherited checklist: "Beacon Release Readiness v0.9"

Neha Sharma sent you this on your first morning with the message: *"We learned from the last programme. Every line has a named owner, every line has a stage, and every gate is already approved. Thursday should be a formality."*

Reproduced exactly as received. ✓ = complete.

#	Item	Stage	Owner	Status	Evidence / notes
1	Business case approved	Intent	Marcus Bell	✓	Board paper, August
2	Pilot sites agreed (Riverside, Hillcrest)	Intent	Dr. Anjali Rao	✓	Executive minutes, September
3	Scope confirmed: referral letters + portal messages	Spec	Neha Sharma	✓	spec.md v3, approved 2 Dec
4	Architecture approved	Spec	Tobias Grant	✓	ADR-009 (September)
5	Vendor contracts and data-processing agreements signed	Plan	Neha Sharma	✓	CTR-311 (Corvid), CTR-312 (MedLink); both DPAs include India data residency
6	Feature plans complete	Plan	Imran Qureshi	✓	Plans in repo
7	Code complete and merged	Build	Imran Qureshi	✓	Build 1.4.2
8	Regression suite green	Build	Jonas Weber	✓	CI run #2291, 27 Nov, build 1.4.2
9	Model validated (96% accuracy)	Build	Jonas Weber	✓	Corvid validation summary
10	Confidence threshold set (0.80)	Build	Dr. Farah Siddiqui	✓	Evaluation report ev-118
11	Clinical safety case signed	Review	Neha Sharma	✓	Hazard log v2 (signed)
12	Code review complete	Review	Imran Qureshi	✓	87 of 87 PRs approved
13	Penetration test passed	Review	Rahul Menon	✓	PT-2026-77: 1 critical finding, fixed; retest passed 30 Nov; signed R. Menon
14	DPIA approved	Review	Grace Liu	✓	DPIA v1, August
15	Load test passed (3× expected volume)	Review	Tobias Grant	✓	LT-1.4.2, 1 Dec: p95 1.8 s at 3× pilot volume
16	Accessibility review	Review	Neha Sharma	N/A	"Backend service"
17	Human review for urgent cases	Release	Priyanka Das	✓	Urgent → nurse queue, 30-minute response
18	Risk register reviewed	Release	Owen Fletcher	✓	R-17 accepted 5 Dec
19	Monitoring and drift dashboard live	Release	Kavya Nair	✓	Grafana "Beacon Drift"
20	Alerts configured	Release	Kavya Nair	✓	#beacon-alerts channel
21	Audit logging enabled	Release	Kavya Nair	✓	log_fields in triage-config.yaml
22	Data retention configured	Release	Platform / Nursing (joint)	✓	365 days

#	Item	Stage	Owner	Status	Evidence / notes
23	Rollback rehearsed	Release	Tobias Grant	✓	20 Nov: 1.4.2 → 1.3.9 in 4 minutes
24	Triage nurses trained	Release	Priyanka Das	✓	E-learning module assigned
25	Contact centre briefed	Release	Sunil Bhat	✓	Email sent 3 Dec
26	Patient communication (AI disclosure) approved	Release	Marcus Bell	✓	Draft with Communications
27	Vendor model-change process agreed	Release	Neha Sharma	✓	Corvid notifies 14 days ahead
28	Network rollout plan (9 hospitals, 11 Jan)	Release	Neha Sharma	✓	Rollout plan v2
29	Board communication	Release	Marcus Bell	✓	Annual report, p. 14
30	Go/No-Go for pilot	Release	Dr. Anjali Rao	☐	10 Dec. "All stage gates already approved (see appendix) — formality."

Appendix notes attached to v0.9

- Stage-gate approvals: Intent ✓ Neha Sharma, 2 Dec · Spec ✓ Neha Sharma, 2 Dec · Plan ✓ Neha Sharma, 2 Dec · Build ✓ Neha Sharma, 2 Dec · Review ✓ Neha Sharma, 2 Dec · Release: at Go/No-Go.
- Threshold note: 0.80 per evaluation ev-118. May be tuned operationally by Engineering to manage nurse workload.
- Ownership note: Technical items owned by Tobias until handover. Operational ownership is joint between Platform and Nursing.
- This checklist covers the pilot and the network rollout. Once the pilot Go/No-Go is passed, the network rollout proceeds per Rollout plan v2.

Part C — Evidence pack

Eight things you found in the repository, the team channel and the systems this morning. Read them against the checklist.

E1. Internal evaluation report — ev-118 (12 November)

Measure	Result	Note
Urgency category accuracy vs. nurse consensus (3,000 Kestrel messages)	0.93	
Under-triage rate: urgent or emergency classed as routine or self-care (all)	2.1%	Safety-critical measure
Under-triage: English-only messages (n = 2,760)	1.6%	
Under-triage: mixed-language or transliterated messages (n = 240)	6.8%	Flagged "review"; logged as risk R-17
Over-triage rate: routine classed as urgent (all)	9.4%	
Messages auto-routed at threshold 0.80	69%	31% go to the nurse queue
Reading level of automated self-care replies (sample of 200)	Grade 11	

Build evaluated: 1.4.0 · Model: corvid-med-3.2 · Threshold: 0.80 · Prepared by Data & AI · Clinical safety review: pending (not reviewed by CSO).

E2. Supplier validation summary

"Corvid-Med v3 triage accuracy 96.2% on CorvidBench-Triage (n = 48,000 primary-care messages from the United States and United Kingdom). Hazard log v2: 18 hazards, all mitigated. Signed: Dr. P. Andrews, Clinical Safety Officer, Corvid AI."

E3. Configuration file in the repository

```
# beacon/triage-config.yaml (commit a41f, 26 Nov)
model: corvid-med-latest        # floating alias; supplier may update
auto_route_threshold: 0.72     # lowered from 0.80 on 25 Nov to reduce nurse queue (IQ)
auto_reply_enabled: true
auto_reply_categories: [routine, self_care]
log_fields: [message_id, category, confidence]
retention_days: 365
alert_channel: "#beacon-alerts"
```

E4. Team channel — #beacon-delivery (selected messages)

Date	From	Message
26 Nov	Imran Qureshi	Lowered the auto-route threshold to 0.72. The nurse queue was too big in the pilot simulation. Eval numbers will be basically the same.
27 Nov	Victor Osei	Regression suite green on 1.4.2 (CI #2291). Note: that suite does not include the model evaluation set.
30 Nov	Corvid AI (supplier notice)	corvid-med-latest will move to corvid-med-3.3 on 15 December. This is a non-material improvement. No customer action is required.
1 Dec	Priyanka Das	Night shift: 2 triage nurses across both pilot sites, and they also cover the ward phone line. In the simulation the urgent queue peaks at about 25 cases between 7 and 10 pm. At about 6 minutes per case that is 75 minutes to clear the peak. The checklist says 30 minutes.
2 Dec	Grace Liu	Reminder: DPIA v1 (approved August) covers referral letters only. Portal messages were added in October, so we need an addendum before patient messages are processed.
3 Dec	Sunil Bhat	When Beacon sends a self-care reply and the patient writes back "it's getting worse", where does that go? Right now it lands in the general inbox, which we answer within 48 hours.
4 Dec	Marcus Bell	Reminder that the board has announced the 14 December pilot in the annual report. We cannot slip.
5 Dec	Neha Sharma	Owen, can you record R-17 (mixed-language under-triage) as accepted? I've accepted it on behalf of delivery so we stay green for Thursday.
7 Dec	Owen Fletcher	Done: R-17 marked "accepted (N. Sharma, 5 Dec)" in the register.

E5. Monitoring

- Dashboard "Beacon Drift" (Grafana): shows the share of messages in each urgency category and mean confidence, compared with a baseline.
- Baseline: the category distribution from CorvidBench-Triage (supplier benchmark). No Kestrel baseline has been loaded.
- No measure of under-triage exists in production. Under-triage can only be found by clinicians reviewing a sample of auto-routed messages; no such sampling is scheduled.
- Alerts post to the #beacon-alerts chat channel. The channel has 14 members; it is not on any on-call rota. Kavya's platform rota is paged only for infrastructure alerts (latency, errors, outages).

E6. Rollback

- Runbook §5 "Rollback": rehearsed 20 Nov in staging. The application container was rolled back from 1.4.2 to 1.3.9 in 4 minutes.
- The model is hosted by Corvid and called through the alias "corvid-med-latest". The runbook does not cover the model version, the prompt version, or switching Beacon off so that all messages go to nurses (manual triage).

E7. Clinical safety case

Hazard log v2 (the document behind v0.9 item 11): 18 hazards, written and signed by Corvid AI's Clinical Safety Officer for CorvidBench deployments. It contains no hazards for automated replies to patients, mixed-language messages, or nurse queue capacity. Dr. Helen Mathews (Kestrel CSO): "I have not been asked to review it."

E8. Other notes behind the ticks

v0.9 item	What you found
Nurse training (item 24)	Learning system report, 7 Dec: 11 of 29 pilot-site triage nurses (38%) have completed the Beacon module.
Patient AI disclosure (item 26)	Communications team, 7 Dec: "Draft wording sent to Legal for review. English only so far."
Contact centre (item 25)	Sunil Bhat: "We got an email with a link to the release notes. No call scripts, no route for patients who say their symptoms are worse."
Vendor change process (item 27)	Contract CTR-311, clause 9.4: Corvid may update the "latest" alias with 14 days' notice. Pinned model versions are available at additional cost. Kestrel selected the "latest" alias.
Architecture (item 4)	ADR-009 is dated 21 September. Portal messages and automated replies were added to scope on 9 October. There is no later architecture review.

Part D — Your task

Complete the **Exercise 2 Answer Workbook**. It has the same structure as the Week 1 workbook, with one addition: Beacon has two rollout phases, so your checklist must show which phase each criterion gates.

Workbook part	What you produce	Scored?
Part 1 · Cover	Candidate id, AI tool used, date, attempt	No
Part 2 · Findings Log	Every weakness in v0.9 that would stop a careful stranger trusting it. Aim for 15 or more	Context
Part 3 · Governance frame	One Accountable person per dimension; a stage-gate map; and what must be true at the **pilot gate** versus the **network gate**	Context
Part 4 · Release-Readiness Checklist v1	25 to 40 criteria. Every row has a stage, a phase, a dimension, a checkable question, evidence, where it is logged, a named owner, an approver, and an honest status as of today with the basis	**Yes**
Part 5 · Readiness statement	Half a page to Dr. Anjali Rao: your decision for 14 December (go, no-go, or go only under stated conditions), what would make it a yes, and what must be true before 11 January	**Yes**
Part 6 · Assumptions	Anything you had to assume that the case does not state	Context

Ground rules

- **Status is as of today, 8 December.** Mark a row Met only if the evidence pack shows it is met. Some things in this case genuinely are met; marking everything Not met is as wrong as marking everything Met.
- **Use only the people in A3.** No teams, no suppliers, nobody who has left, nobody invented.
- **Policies are real constraints.** If your checklist would break AIP-2, CSS-1, RP-1 or PCS-3, it is not ready.
- **Messages in the team channel are not evidence.** A statement that something is fine, accepted or non-material is a claim. Ask who has the authority and where it is recorded.
- **Your decision must be one the sponsor can act on.** "It depends" is not a decision. "Go only if…" is, provided each condition is specific, owned and checkable.
- **Write it so the evaluator can quote it.** The evaluator scores only what it can quote from your workbook. Polish earns nothing.

How you will be scored

Rubric line	What the evaluator looks for
Governance Workflow Design (core skill)	All five dimensions, each with one named Accountable owner who can actually be accountable

Rubric line	What the evaluator looks for
AI Governance Policy Design (core skill)	Confidence threshold, human-in-the-loop trigger and model drift monitor as three separate, fully specified rows
Stage-Gate & Artefact-Chain Literacy (core skill)	Criteria mapped to the six stages and to the right phase, not one flat gate
Trust & Governance Design (competency, target Level 3)	Trust comes from visible, checkable evidence at each stage, not from a sign-off

Weekly score = core skills average × 60% + competency × 40%. Pass = 50. The evaluator runs twice; every score cites your words; missing scores 0; a visible attempt that falls short scores 25.

Part E — Submission

Submit your completed Exercise 2 Answer Workbook, named **UST_AITPM_Week1_Ex2_Workbook_<your candidate id>**, through the process your facilitator gives you. Do not paste screenshots in place of text.